



Guideline

DISCHARGE ANALGESIA PRESCRIBING GUIDELINE

Scope

Site	Service/Department/Unit	Disciplines
SCGH and OPH	Organisation Wide	All Staff, Medical, Nursing, Pharmacy

HIGH RISK MEDICATIONS

Narcotics (opioids) and sedating analgesics like pregabalin are high-risk medications.

For support, contact the Acute Pain Service (APS) via Pager 4120 (Mon-Fri 0800-1700, Sat 0800-1200). After hours, reach the Anaesthesia Registrar on Pager 4823.

Introduction

This guideline outlines the procedures and responsibilities of individuals and the health service organisation in relation to analgesia prescribing for discharge. It ensures the delivery of a uniform, integrated multidisciplinary approach with management that is consistent with best practice in accordance with the Australian Commission on Safety and Quality in Health care (ACSQHC): Opioid Analgesics Stewardship in Acute Pain. .

Guideline Principles.

The "[Preventing Progression to Persistent Pain](#)" toolkit should be used to support local opioid stewardship programs, promoting patient-centred education throughout perioperative continuum of care to prevent progression to persistent pain. It includes [frameworks](#) and [teaching tools](#). Current acute post-op pain guidelines recommend multimodal analgesia and non-pharmacological therapies. As more patients are discharged with opioid prescriptions, safe pain management must involve:

1. Risk assessment before discharge
2. Individualised analgesia plans
3. Patient education on opioid use and disposal
4. GP communication and follow-up

Opioids pose risks like respiratory depression, falls, and persistent use. Over-prescription leads to unused opioids in the community, contributing to rising opioid-related harms, including deaths from prescription opioids surpassing heroin fatalities.

1.0 CONSIDERATIONS

When prescribing analgesia for discharge, utilise the [pain assessment and management algorithm](#). Expanded criteria can be found [here](#).

This should be done in conjunction with an appropriate risk screen. [The Opioid Risk Tool](#) (ORT) is a brief, self-report screening tool designed for use with adult patients in primary care settings to assess risk for opioid abuse among individuals prescribed opioids for treatment of chronic pain.

2.0 GENERAL RECOMMENDATIONS

CHECK FIRST

Check for APS or anaesthetist discharge plans within the medication chart and patient notes (look for APS Discharge Analgesia Plan green form). **This specialist advice should be followed at all times.**

All patients prescribed analgesia on discharge should be advised to visit their GP **WITHIN** one week after discharge.

Avoid the use of TRAMADOL in patients taking TWO or MORE serotonergic agents to prevent potential of serotonin syndrome, unless advised by specialist pain management service

For general [Analgesia prescribing tips](#) see [here](#)/

For [Discharge analgesia tips](#) see [here](#).

Other

- Simple analgesia i.e. paracetamol and/or ibuprofen should be prescribed as first line for most patients.
- Consider prescribing aperients for the duration of prescribed opioid analgesia to avoid patient constipation e.g. docusate & senna 50/8mg x 1-2 BD PRN.
- Discharge analgesia as documented by the Anaesthetist or APS
- **Do NOT prescribe:**
 - Sustained release opioids
 - Immediate release opioids for MORE than 5 days expected use
 - Gabapentin/pregabalin or ketamine without specialist advice and appropriate follow up.
- Encourage your patient to schedule a review within 5 days with their usual prescriber.

DO NOT prescribe without seeking advice from the APS

- **Opioids for discharge if the patient has not required any doses of opioids in the 24 hours prior to discharge**
- Clonidine on discharge unless authorised by APS
- Ketamine lozenges or wafers unless authorised by APS
- Fentanyl lozenges

3.0 PRACTICAL POINTS

Drug Interactions

- Depending on the type of analgesia prescribed there may be drug-drug or drug-disease interactions. Please check prior to prescribing and speak with your ward pharmacist for advice if needed.

Adverse Effects

- Refer to AusDlor consult your ward pharmacist for the individual side effect profile for each medication. Please consider prior to prescribing.

Use in Pregnancy

- Always consult expert resources such as the [KEMH Clinical Guidelines for Obstetrics and Gynecology](#).

Use in Breastfeeding

- Always consult expert resources such as the [KEMH Clinical Guidelines for Obstetrics and Gynecology](#).

Instructions to Patients

The below information should be communicated to the patient:

- Take these medicines only as prescribed – do not take extra pain medication if it has not been prescribed including over the counter medicines.
- Expect your use of pain medications to decrease as healing takes place.
- It is not expected for you to require long-term pain medication after this surgery.
- Please contact your GP if your pain is not controlled with your current medication.
- If you are sleepy from the medications, do not drive or operate machinery.
- Alcohol use whilst taking strong pain medication increases the risk of sedation – speak to your Doctor or Pharmacist if you are not sure.
- If you experience unexpected ongoing pain or any side effects from the medications, please make an appointment to see your GP.
- Please take any leftover pain medicines that you are not taking to your pharmacist for disposal. It is dangerous and illegal to take pain medications that have been prescribed to a different person.

Instructions to GP

Information about discharge analgesia and weaning plans should be communicated to the patients GP:

- If a referral has not been arranged and you think pain will be an ongoing problem, please fax a referral to the pain management department on 6457 3481

Related Documents

- WATAG. [Recommendations for prescribing analgesia on discharge following surgery or acute injury](#). 2017.
- [Medicines Management Policy and Procedure](#). SCGOPHCG Updated 2024
- <https://www.safetyandquality.gov.au/standards/clinical-care-standards/opioid-analgesic-stewardship-acute-pain-clinical-care-standard>

KEY LEGISLATION & ACTS

- [Statewide Medicines Formulary Policy MP 0077/18](#)
- Medicines and Poisons Act 2014
- Medicines and Poisons Regulations 2016
- Health Services Act 2016

STANDARDS

NSQHS Standards:



National Standard 1: Clinical Governance



National Standard 2: Partnering with Consumers



National Standard 4. Medication Safety



National Standard 6. Communicating for Safety

see <https://www.safetyandquality.gov.au/our-work/assessment-to-the-nsqhs-standards/nsqhs-standards-second-edition/>

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Standards Applicable:	Insert relevant NSQHS Standards (Version 2):  EQulPNational Standards, also any other national standards that are relevant eg. ACORN, ANZCA, MAGNET etc. NSMHS: 1, 2, 3, 4, 5, 6, 7, 8, 9, 10.1, 10.2, 10.3, 10.4, 10.5, 10.6,				

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

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